MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 9 OF 28

MFT 709

Group Therapy Techniques

15 teaching weeks 5 practicals 1 figure

SEMESTER TWO

1.71 units Prerequisite: MFT 706

> Groups are the most efficient and the most unpredictable clinical format.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 709 Group Therapy Techniques

MSc Marriage and Family Therapy Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	**Group Therapy Techniques**	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Groups are the most efficient and the most unpredictable clinical format. This course teaches you to design a group, build a culture, work with the process rather than only the content, manage the difficult members, and co-facilitate without performing a marriage in front of the members.

How to use this book

A group cannot be learned from a book alone, but it can be ruined without one. Each week gives you the theory, a facilitation script, and a hazard. The practical sessions in this course run as a real group: you will be members before you are leaders, and you will write about both.

1. Course description

Group therapy is a distinct modality with its own theory of change: members help each other, and that mutual aid is the treatment. Family therapists use groups for couples (multiple couples in one group), parents, adolescents, men who have used violence, people in recovery, the bereaved, and children.

This course covers group design, composition, stages of development, therapeutic factors, process observation, difficult members, co-facilitation, ethics and evaluation. It is grounded in Yalom's interpersonal model and in systemic thinking about the group-as-a-system.

2. Learning outcomes

#	Outcome	Assessed
LO1	Design a group: purpose, membership, size, duration, setting	Group proposal
LO2	Explain and use Yalom's therapeutic factors	Exam
LO3	Identify and work with group process (here-and-now)	Observed facilitation
LO4	Manage difficult members and group crises safely	Viva, observed
LO5	Co-facilitate effectively, including managing disagreement	Peer and tutor rating
LO6	Screen and prepare members ethically	Screening role-play
LO7	Evaluate group outcomes	Evaluation plan

3. Assessment

Component	Weight	Due
Group design proposal (2,000 words)	25%	Week 6
Process reflection log (as member, 6 entries)	20%	Fortnightly

Component	Weight	Due
Observed co-facilitation (30 min)	30%	Weeks 12 14
Examination	25%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why groups work: mutual aid and the social microcosm	First group session (as member)
2	Types of group in family therapy	Observation of a live/recorded group
3	Design: aims, size, duration, open vs closed	Design clinic
4	Screening, consent and preparation	Screening role-play
5	Yalom's therapeutic factors	Factor-spotting exercise
6	**Group proposal due**; group stages I: forming and storming	Process observation
7	Group stages II: norming, performing, adjourning	Leader tasks by stage
8	The here-and-now: working with process	Process commentary drill
9	Difficult members: monopoliser, silent, hostile, help-rejecting	Structured rehearsals
10	Conflict, scapegoating and subgrouping in groups	Live intervention
11	Co-facilitation: division of labour, disagreement, repair	Co-therapy pairs
12	**Observed co-facilitation begins**	Assessed
13	Groups for couples and for parents	Design adaptation
14	Ethics, confidentiality and boundaries in groups	Incident analysis
15	Evaluation and endings	Outcome plan

Week 1 Why groups work

The core claim. In a group, members are simultaneously helpers and helped. This reciprocal process Yalom called it mutual aid produces learning that a one-to-one relationship cannot, because it is horizontal rather than vertical. The leader is not the only source of help; the membership is.

The social microcosm. A group is a small society in which each member eventually behaves as they do outside: the man who must control, the woman who apologises, the adolescent who deflects with humour. The group then becomes a laboratory in which those behaviours are noticed, felt, and changed in vivo.

Systemic framing. A group is a system: it has boundaries (who is in, who is out), roles, norms, a developmental history, and homeostatic mechanisms that resist change.

Everything you learned about families applies. The scapegoat in a group is playing the same structural role as the identified patient in a family.

> **PRACTICAL**
> Begin the experiential group this week. The group runs for the semester with two student co-leaders rotating. Write your first reflection on what it felt like to be a member.

Week 2 Groups in family therapy practice

Group type	Members	Typical aims
Couples group	4 6 couples	Communication skills, shared normalisation, vicarious learning
Parents group	6 10 parents	Behaviour management, support, reducing isolation
Adolescent group	6 8	Identity, peer feedback, social skills
Men's behaviour change	8 12	Accountability, gender norms, non-violence
Recovery group	8 12	Sobriety support, family reintegration
Bereavement group	6 10	Normalising grief, meaning-making
Psychoeducational	10 20	Information plus discussion
Multi-family group	4 6 families	Family family learning (strong evidence in psychosis and eating disorders)

Multi-family groups deserve particular attention in this programme: they combine systemic work with mutual aid, are cost-effective, and have good evidence in severe mental illness, eating disorders and adolescent conduct problems.

Week 3 Design

Eight design decisions. Each should be justified in your proposal.

1. **Aim** what change, for whom, measured how?
2. **Membership** who is included, who excluded, and on what criteria?
3. **Size** 6 10 for interactional groups; larger for psychoeducation.
4. **Duration and frequency** 8 16 sessions weekly is typical; state the rationale.
5. **Open or closed** closed builds cohesion; open tolerates attrition.
6. **Setting and materials** circular seating, privacy, accessibility.
7. **Leadership** solo or co-facilitated (co-facilitation is strongly preferred for beginners).
8. **Evaluation** pre/post measures, attendance, satisfaction, and a follow-up.

Composition traps. Mixing victims and perpetrators of violence in one group; mixing adolescents and adults; including two people in a current conflict (e.g. two co-wives, or a

couple in the same parents' group); including a member who knows another member socially without discussing it in advance.

> **PROPOSAL (25%)**
> A 2,000-word design document: population, aims, evidence base, membership criteria, screening protocol, session-by-session outline, facilitation plan, risk protocol, evaluation plan, budget.

Week 4 Screening, consent and preparation

Screening protects the group as much as the individual. Screen for: fit with the group's aims, current risk, ability to tolerate group feedback, concurrent individual therapy, and practical commitment.

The screening interview (30 minutes).

1. What brings you here, and what do you hope for?
2. What has helped before? What has not?
3. Have you been in a group before? How was it?
4. How do you usually handle conflict and criticism?
5. Is there anyone in the group you know? (Show the member list where appropriate.)
6. Are you currently safe? (Risk screen.)
7. Are you willing to keep confidentiality, knowing it cannot be guaranteed?

Consent must be specific. Explain that confidentiality is a group norm but not a guarantee; that members may be upset by others; that the leaders will break confidentiality only in the circumstances described; and that attendance and punctuality are part of the contract.

Preparation improves outcomes. Tell the member what to expect, what will be asked of them in the first session, and how to introduce themselves. Pre-group preparation reduces drop-out measurably.

> **PRACTICAL**
> Screening role-play with a volunteer who is ambivalent and has been pressured to attend. Practise: exploring ambivalence, explaining the frame, and declining a poor fit without rejecting the person.

Week 5 Yalom's therapeutic factors

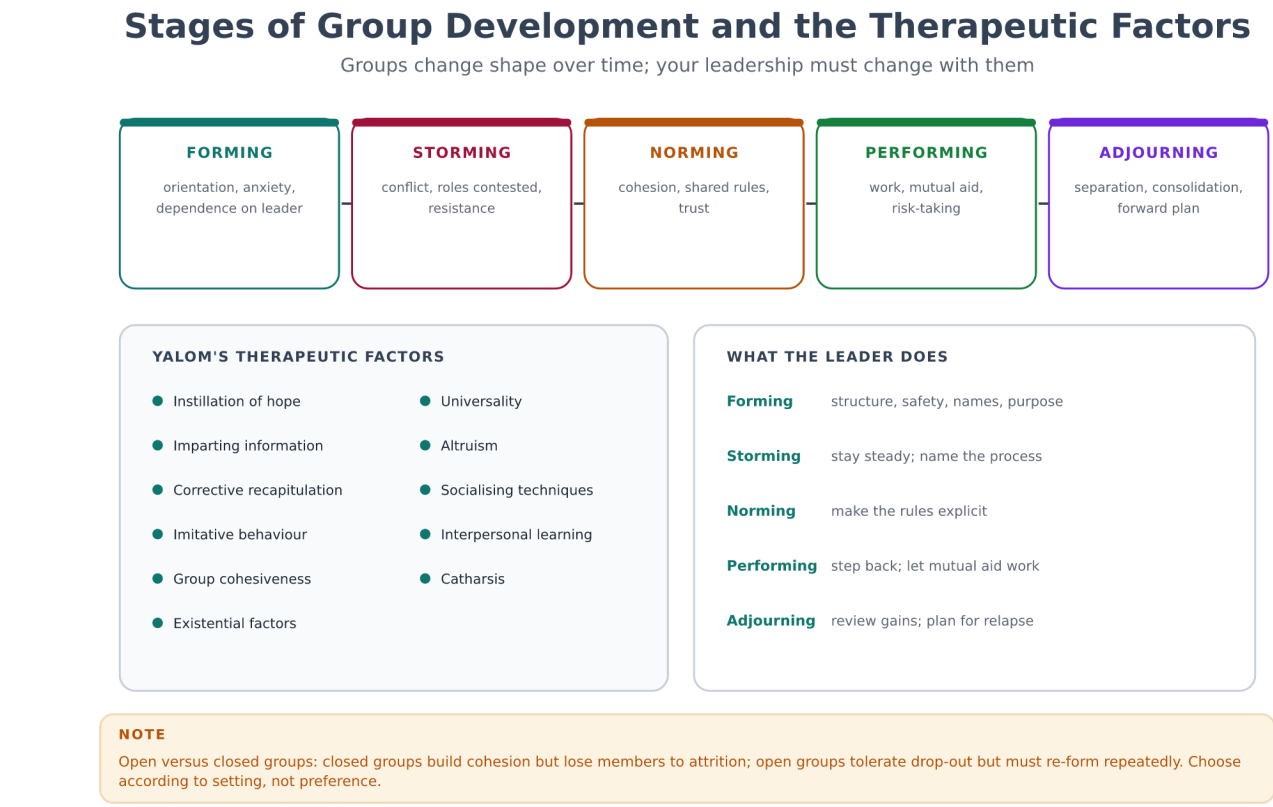


Figure 5.1 Group stages, therapeutic factors and the leader's changing task

Factor	What it looks like	How the leader strengthens it
Instillation of hope	A member describes recovery	Ask for specifics; keep the member connected to the group
Universality	"I thought I was the only one"	Name the shared experience explicitly
Imparting information	Advice, psychoeducation	Redirect advice-giving into dialogue
Altruism	A member helps another	Point out the help; let the helper be seen
Corrective recapitulation	Old family patterns reappear	Interpret the pattern in the here-and-now
Socialising techniques	Learning social feedback	Model and reinforce it
Imitative behaviour	Modelling the leader or a member	Use yourself deliberately
Interpersonal learning	Feedback changes behaviour	Make the group the site of the experiment
Group cohesiveness	Belonging, attendance, care	Protect the frame; address absences
Catharsis	Emotional release	Contain it; link it to meaning
Existential factors	Responsibility, mortality, isolation	Allow the conversation; do not rush to technique

The beginner's error: chasing catharsis. Tears without integration produce relief for a day and no change. The sequence is: **express understand experiment consolidate**.

Week 6 Stages I: forming and storming

Forming. Anxiety, politeness, dependence on the leader, search for structure. Members ask: "What are we supposed to do? Is this allowed?"

Leader task: provide structure and safety, model disclosure at an appropriate level, establish norms (punctuality, confidentiality, one person at a time, no advice without asking), and name the anxiety.

Script: "Most groups start quietly. Part of what we will do is notice what we are all avoiding."

Storming. Conflict, role competition, resistance, criticism of the leader, subgrouping. This is not failure; it is the group testing whether it can survive difference.

Leader task: stay steady, do not defend, name the process, and let the group negotiate. Avoid solving it for them.

Script: "There is something happening between the two of you that the rest of us are watching. Can we look at it together rather than settle it?"

> **PRACTICAL**
> Process observation. Two observers code the session for: who speaks, who is interrupted, who is silent, and three moments of process (as opposed to content). Feed back to the group in the last ten minutes.

Week 7 Stages II: norming, performing, adjourning

Norming. Cohesion, shared rules, mutual trust, humour. Attendance stabilises.

Leader task: consolidate the norms explicitly; use the cohesion to do deeper work.

Performing. The productive stage: mutual aid, confrontation with care, risk-taking, insight.

Leader task: step back. Your best sessions are the ones in which you spoke least and the group worked. Redirect all content to the members: "What does the group think?"

Adjourning. Ending, loss, consolidation, transfer to life outside.

Leader task: review gains, name the loss, have each member say what they gave and received, and make a forward plan. Do not let the group end by attrition.

Termination script: "We have four sessions left. I want us to use them for two things: what each of you has learned, and what each of you will do when the group is not here."

Week 8 The here-and-now

Two layers. Content is what the group is talking about; process is what is happening between the people while they talk about it. Groups change through process.

The four-step process commentary.

1. **Notice** something concrete: "Every time Ada speaks about her marriage, Femi looks at the clock."
2. **Name it to the group, not about the person:** "I notice something happening here."
3. **Ask the group to look:** "Does anyone else notice it? What do you think it is about?"
4. **Return it to the relationship:** "Femi, what is happening for you when Ada speaks?"

Timing rules. Do not process-comment in the first two sessions (no safety yet). Do not do it to demonstrate skill. Do it when the same pattern has occurred at least twice.

Hazards. Process commentary can feel like exposure. Use the group as the buffer ("does anyone else "), avoid naming individuals as specimens, and never interpret someone's process before you have checked it with them.

> **DRILL**
> Ten minutes of "process-only": the group may only speak about what is happening here, not about events outside the room. Debrief how uncomfortable it was.

Week 9 Difficult members

Member type	Behaviour	What it protects	Intervention
Monopoliser	Talks at length; fills silence	Anxiety, fear of invisibility	"I want to bring others in; you may find that hard." Set a turn-taking norm
Silent member	Says nothing; attends	Fear of exposure, cultural deference	Low-demand invitation; do not force; check in privately
Hostile/devaluing	Attacks the leader or a member	Shame, previous betrayal	Name impact; set the boundary; explore what it protects
Help-rejecting complainer	Asks for help then rejects it	Control, fear of change	Move from advice to the function: "What would it cost you to accept help?"
Group mascot	Defuses everything with humour	Avoids grief	"Your jokes are welcome. I also want to know what is underneath this one."
Scapegoat	Becomes the group's problem	Carries disowned material	Protect; interpret the group's function in selecting them
Subgroup	Two or three form a bloc	Safety in numbers	Bring it into the open; do not let it be secret

The principle. Every difficult behaviour is a solution the member has found for a problem. Your task is to understand the problem, not to eliminate the behaviour by force.

Week 10 Conflict, scapegoating and subgrouping

Escalation in groups is faster than in dyads because of contagion. Interrupt earlier than you would in a family session.

The scapegoat sequence (learn to spot it): a member is different the group's anxiety attaches to them the group pressures them the leader is invited to condemn them if the leader agrees, the group learns that difference is dangerous.

Leader intervention: name the process, protect the member, and give the group the work: "I notice we have all become very interested in what Femi is doing wrong. What is it about him that makes us comfortable focusing there?"

Subgroups. Two members who meet outside, or a bloc that votes together. Name it without accusation: "I am aware that some of you talk outside. That is allowed, and it affects what happens here. What shall we do about it?"

Week 11 Co-facilitation

Why co-facilitate. Two sets of eyes, two relational styles, a model of a working relationship, and protection when one of you is triggered.

Division of labour decide before every session.

- Who opens and closes?
- Who tracks process, who tracks content?
- Who handles risk or a member leaving the room?
- How will you signal a disagreement in the moment?

Managing disagreement in front of the group. Never contradict your co-leader without acknowledging it. Options: (a) defer and discuss after; (b) name the difference openly and model resolution: "I see it differently, and I want the group to hear both." Naming it well is a gift; naming it badly is a rupture.

The classic co-therapy failure: the couple dynamic. Two co-leaders who replicate a parent child or pursue withdraw pattern in front of the group. The group will notice and may act it out. Take it to supervision immediately.

> **PRACTICAL**
> Co-therapy pairs plan a session in ten minutes using the division-of-labour checklist, then facilitate with the observed group. The observer records every moment the pair were out of sync.

Week 12 Observed co-facilitation

Thirty minutes of an experiential group, co-facilitated, assessed against: safety and structure; use of process; inclusion of silent members; containment of difficult behaviour; co-leader coordination; and closing.

Week 13 Couples and parents groups

Couples group. Four to six couples. Strengths: normalisation ("other couples have this too"), vicarious learning, and reduced shame. Risks: public conflict, competition, and one partner feeling ganged up on.

Structure: 15 minutes check-in (one sentence per person), 20 minutes teaching, 30 minutes exercise (couples work together while leaders circulate), 15 minutes group debrief. Never ask a couple to do a deep vulnerability exercise in front of others before they have done it alone with you.

Parents group. Strengths: reducing isolation, shared problem-solving. Risks: one parent becoming the group's expert, and advice-giving replacing reflection.

Structure: check-in, one skill, one application, one commitment. Keep advice horizontal: "Who has tried something that worked?"

Week 14 Ethics and boundaries in groups

Issue	Standard
Confidentiality	Explain it is a norm, not a guarantee; revisit when a member leaves
Screening	Never skip it; document it
Dual relationships	Members who know each other; leaders who know members socially
Touch	Discourage between members; use only with explicit norms
Out-of-group contact	Agree a policy; do not become a member's confidant
Recordings	Separate consent from each member; a member may withdraw at any time
Risk disclosure	Leaders follow their duty of care; tell the group if confidentiality must be broken

The most common ethical complaint in groups is not dramatic: it is a member learning that something they said was repeated outside. Prevention: explicit norms, a written agreement, and a leader who models discretion.

Week 15 Evaluation and endings

Minimum evaluation package. Attendance and drop-out (with reasons), a pre/post measure of the target problem, a session-by-session satisfaction rating, and a follow-up at 8 12 weeks.

Writing up. Report the design, the actual membership, the adaptations you made, the outcome and the limitations. A well-reported small group is a contribution; an unreported one is a private experience.

Glossary

Cohesion the group's sense of belonging and mutual commitment; a precondition for productive work. **Group-as-a-whole** the group considered as a single system with its own dynamics. **Here-and-now** the immediate interpersonal process in the room, as distinct from outside content. **Mutual aid** the reciprocal helping that occurs between members. **Process** the relational layer of communication, as distinct from content. **Scapegoating** the group's displacement of anxiety onto one member. **Social microcosm** the group as a small society in which members' characteristic patterns appear. **Subgrouping** the formation of alliances within the group. **Therapeutic factor** a mechanism of change identified by Yalom (e.g. universality, cohesiveness).

Self-assessment

Multiple choice

1. Mutual aid refers to: (a) the leader helping members (b) members helping each other (c) financial support (d) group cohesion

2. Scapegoating in a group usually functions to: (a) help the scapegoat (b) absorb the group's anxiety (c) improve cohesion (d) test the leader

3. Pre-group preparation: (a) is optional (b) reduces drop-out and improves outcome (c) increases anxiety (d) is only for research

4. The leader's main task in the performing stage is to: (a) direct more (b) step back and let mutual aid work (c) introduce new rules (d) end the group

5. "Here-and-now" work means: (a) discussing childhood (b) working with immediate in-room process (c) using mindfulness (d) homework review

6. Confidentiality in groups is: (a) guaranteed (b) a norm that cannot be guaranteed (c) irrelevant (d) the leader's legal duty only

7. A closed group: (a) admits new members (b) has fixed membership (c) has no rules (d) is always shorter

8. Catharsis without integration typically produces: (a) lasting change (b) temporary relief (c) increased cohesion (d) no effect at all

9. The best composition decision when two members are in an active conflict is to: (a) include both to work it out (b) avoid placing them in the same group (c) seat them together (d) ask them to sign a contract

10. Parallel process in co-facilitation means: (a) the leaders take turns (b) the leaders' relationship replicates a family pattern (c) members copy the leaders (d) two groups run at once

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. List five design decisions and the rationale for each.
2. Name five therapeutic factors and one leader behaviour that strengthens each.
3. Give the four steps of process commentary with an example.
4. How would you intervene with a monopoliser without shaming them?
5. What are the four elements of a group evaluation plan?

Essay (choose one, 1,800 words)

1. Design a couples group for eight distressed couples, justifying every design decision.
2. "The group is the treatment, not the leader." Discuss with reference to mutual aid and process.
3. Analyse a difficult moment in your experiential group and set out what you would do differently as leader.

Further reading

- Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books.
- Bion, W. R. (1961). Experiences in Groups. Tavistock.
- Burlingame, G. M., Strauss, B., & Joyce, A. S. (2013). Change mechanisms and effectiveness of small group treatments. In Bergin and Garfield's Handbook.
- Asen, E., & Scholz, M. (2010). Multi-Family Therapy: Concepts and Techniques. Routledge.

Next: MFT 710 Couples Therapy